Inpatient Unit Palliative Care Clinical Assessment Form

PCOC palliative care outcomes collaboration

Patient Addressograph Here

ONGOING ASSESSMENT:

Complete at initial contact; at each contact; at phase change; and prior to separation If not assessed, enter N/A

Year:	Date													
	Time													

Clinician-assessed **Is delirium present?** (Yes / No)													
Guide to clinical response	If delirium is suspected, confirm with a validated tool. Consider patient's goal of care and plan care accordingly. 4AT If terminal delirium/terminal agitation confirmed by Senior clinician no need to complete 4AT												

PATIENT SCORED

Edmonton Symptom Assessment System (ESAS-PCOC)

Patient (or proxy) reported symptom and problem **severity** in the last 24 hours

Pain (0 = No pain, 10 = Worst possible)													
Tiredness (lack of energy) (0 = No tiredness, 10 = Worst possible)													
Drowsiness (feeling sleepy) (0 = No drowsiness, 10 = Worst possible)													
Nausea (0 = No nausea, 10 = Worst possible)													
Appetite* (0 = **Best** appetite, 10 = Worst possible)													
Shortness of breath (0 = No shortness of breath, 10 = Worst possible)													
Constipation (0 = No constipation, 10 = Worst possible)													
Sleep* (0 = **Best** sleep, 10 = Worst possible)													
Depression (feeling sad) (0 = No depression, 10 = Worst possible)													
Anxiety (feeling nervous) (0 = No anxiety, 10 = Worst possible)													
Wellbeing (how you feel overall)* (0 = **Best** wellbeing, 10 = Worst possible)													
Other symptom (D = Dry or sore mouth; I = Itch; S = Swallowing; Ot = Other; N = No other symptom)													
Other symptom score (0 = Absent symptom; 10 = Worst possible)													
Guide to clinical response	0 to 3: Anticipate & monitor, modify as required			4 to 7: Review, intervene & refer as required					8-10: Urgent action				

ESAS-PCOC **Rated by** (P=Patient, F=Family/carer, C=Clinician, O=Other)													

Please note 0 represents 'best' for appetite, sleep and wellbeing.

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Year:	**Date**													
	Time													

CLINICIAN - ASSESSED

Resource Utilisation Groups – Activities of Daily Living (see scoring guide)

Bed mobility (1, 3, 4, 5)													
Toileting (1, 3, 4, 5)													
Transfers (1, 3, 4, 5)													
Eating (1, 2, 3)													
Total RUG-ADL (4 to 18)													
Guide to clinical response	If total score is 10 or more, consider equipment needs, case/family conference, staff requirements, carer burden, referral												

Australia-modified Karnofsky Performance Status (see scoring guide)

AKPS (10 to 100)													
Guide to clinical response	If score 60 or lower, consider referral as appropriate. If score 40 or lower, consider also equipment needs, staffing, falls risk, pressure injuries, and carer burden												

Palliative Care Problem Severity Score (0 = absent 1 = mild 2 = moderate 3 = severe)

Pain													
Other symptoms													
Psychological / spiritual													
Family / carer													
Guide to clinical response	0 to 1: Anticipate & monitor, modify as required				2: Review, intervene & refer as required					3: Urgent action			

Palliative Care Phase

	Phase Type												
	Stable (S)	**Unstable (U)**	**Deteriorating (D)**	**Terminal (T)**									
Definition	Adequate symptom and problem control. No change in care plan required.	Urgent change in care plan or emergency treatment required to address severe needs.	Non-urgent change in care plan required to address increasing needs.	Death likely in a matter of days.									
Guide to clinical response	Continue current care. Anticipate & monitor for changing needs.	Urgent action.	Review & intervene; Refer as required.	Adjust care plan to address end of life needs.									

Clinician initials													

DEFINITIONS & SCORING GUIDE

Refer to PCOC Clinical Manual for the complete definitions

Carer Availability

- *Definition of Carer:* paid and/or unpaid person who provides care, support and assistance. May include family, friends, a neighbour, or a paid nursing/carer service. A carer can be different from next of kin/person responsible.
- *Available* means able to support the patient if required.

Delirium: Clinician-assessed presence of delirium in the last 24 hours

- *Definition:* an acute, fluctuating disturbance in attention, awareness, and cognition that often reflects an underlying medical, psychological, or pharmacological cause.
- *Characteristics:* disturbances in consciousness, attention, cognition and perception that develop over a short period of time (usually hours to a few days). Patients with delirium may be agitated and restless (hyperactive delirium), or quiet and withdrawn (hypoactive delirium). Patients may also move between these two subtypes (mixed delirium). The onset of delirium is quick (over hours to a few days) in comparison to dementia.

Edmonton Symptom Assessment System (ESAS-PCOC): Patient-reported severity of symptoms and problems in the last 24 hours

- *Best-practice:* patient rates their own symptoms, which is key to delivering person-centred and value-based care.
- *How to use this tool:* Elicit ratings from patient through conversations to understand the symptoms from the patient's perspective, then conduct a more in-depth clinical assessment as needed. Provide intervention/escalation if necessary.
- If a patient is unable to provide a score, e.g. due to confusion, unconsciousness, communication difficulties, then a proxy can be used. When using a proxy, consider a family/carer first. If family/carer unavailable, then a clinician may score.
- Patient information leaflet 'Let's talk about you care' is available to support clinician and patient discussion.

Resource Utilisation Groups - Activities of Daily Living: Clinician assessment of functional dependence over a 24 hour period

Scoring guide:

For Bed Mobility, Toileting & Transfers

1	Independent or supervision only
3	Limited physical assistance
4	Other than two-persons physical assist (1 person & aid)
5	Two or more persons physical assist

For Eating

1	Independent or supervision only
2	Limited assistance
3	Extensive assistance / total dependence / tube fed

Australia-modified Karnofsky Performance Status: Clinician assessment of performance relating to work, activity and self-care over a 24 hour period

Scoring guide:

100	Normal; no complaints; no evidence of disease
90	Able to carry on normal activity; minor signs or symptoms of disease
80	Normal activity with effort; some signs or symptoms of disease
70	Cares for self; unable to carry on normal activity or to do active work
60	Able to care for most needs; but requires occasional assistance
50	Considerable assistance and frequent medical care required
40	In bed more than 50% of the time
30	Almost completely bedfast
20	Totally bedfast and requiring extensive nursing care by professionals and/or family
10	Comatose or barely rousable

Palliative Care Problem Severity Score: Clinician assessment of problems in the last 24 hours

Global assessment of four palliative care domains to summarise palliative care needs and plan care

- *Pain*: overall severity of pain problems for the patient
- *Other Symptoms:* overall severity of problems relating to one or more symptoms other than pain
- *Psychological/Spiritual:* severity of one or more problems relating to the patient's psychological or spiritual wellbeing
- *Family/Carer:* severity of one or more family/carer problems associated with a patient's condition or palliative care needs. Written, verbal or observational information can be used to assess needs if family/carer are not present

Palliative Care Phase Type: A classification of clinically meaningful periods in an individual's care trajectory

The palliative care phase is determined by a holistic clinical assessment which considers all the above assessments.

Stable (S)	Unstable (U)	Deteriorating (D)	Terminal (T)
Adequate symptom and problem control. No change in care plan required	Urgent change in care plan or emergency treatment required to address severe needs	Non-urgent change in care plan required to address increasing needs	Death likely in a matter of days. Adjust care plan to address end of life needs.